

EMS Scenarios - Verified by EMT

Scenario 1 - Original - Protocol Opioid Overdose

Responded to an unconscious male in his 20s found in a gas station bathroom, showing snoring respirations and cyanosis but with a tachycardic pulse. The attendant had given him the bathroom key about 15 minutes earlier, growing concerned over the delay. Found the patient with drug paraphernalia, indicating possible opioid overdose.

On assessment, the patient was GCS 3, unconscious, with obstructed breathing. Initial vitals showed a pulse of 110 bpm, respiration at 4/min, **BP at 140/70 mmHg**, and SPO2 at 86%. Administered a Nasopharyngeal Airway and must ventilate with a BVM.

Ventilating the patient with a BVM

Ventilating with a BVM: Start Action

...

End Action

Following up with 2mg intranasal naloxone, suspecting opioid involvement.

Post-naloxone, the patient regained full consciousness with improved vitals: GCS 15, pulse 98 bpm, **BP 100/69 mmHg**, respiration 18/min, and SPO2 100%. The patient admitted to substance abuse but denied other medical history. Transported for further care after significant improvement from [the](#) initial presentation.

Verified and Realistic - Scenario 1 - Protocol Opioid Overdose

En route

Dispatch: Medic 425 is responding to an unconscious male in his 20s found in a gas station bathroom. He has a pulse and shallow breathing.

On scene

Attendant: Please, help over here! The bathroom is in the back of the store, let me show you. I gave the man a bathroom key about 15 minutes earlier and he never came out. I went in to check on him and found him on the floor.

[Attendant escorts the Paramedic and EMT to the bathroom to find the man lying on the floor]

EMT: [While shaking the shoulders of the man] Sir! Can you hear me!

Patient: [No response]

EMT: [While checking for a pulse] He's got a pulse and is breathing.

Paramedic: [Checking pupils] Pupils are constricted Get the Narcan.

Paramedic: [Administers narcan intranasally]

EMT: [Starts ventilating the patient with the BVM]

Paramedic: [Establishes an IV and administers more Narcan through the IV]

Patient: [Starts to wake up]

EMT: [Stops ventilating the patient] Sir, can you hear me?

Patient: Ahhhhh.....where am I?

EMT: We are in the Shell gas station bathroom. We found you here and you were unconscious. We need to take you to the hospital.

EMT and Paramedic: [Help the patient onto the stretcher and into the ambulance]

EMT: [Takes vitals in the back of the ambulance which are as follows: GCS 15, pulse 92 bpm, BP 110/65 mmHg, respiration 14/min, and SPO2 98%.]

EMT and Paramedic: [Transport the patient to the hospital without incident]

Scenario 2 - Gemini Generated - Original

Responded to a suspected opioid overdose. Found an unconscious, cyanotic female in her 30s with agonal respirations. Initial assessment revealed GCS 3, unresponsive, with labored breathing. Vitals showed a pulse of 100 bpm, respiration at 6/min, BP at 120/75 mmHg, and SPO2 at 72%.

Ventilating the patient with a BVM

Ventilating with a BVM : Start Action

...

End Action

Administered 2mg intranasal naloxone. Post-naloxone, the patient regained consciousness with improved breathing.

Verified and Realistic - Scenario 2 - Protocol Opioid Overdose

En route

Dispatch: Medic 415 is responding to a female in her 30s found unconscious at home. Potential cardiac arrest.

On scene

EMT and Paramedic: [Enter the patient's home through the unlocked front door] Hello, we are here for the 911 call!

Patient's mother: We are up here!

[EMT and Paramedic run up the stairs to find the patient unconscious on her bedroom floor with the patient's mother administering CPR]

EMT: [Approaches the patient] Ma'am, do you mind moving out of the way?

EMT: [Checks patient for a pulse] She has a pulse and is breathing.

Paramedic: [Over the radio to dispatch] Albemarle, Medic 415 denies the code. The patient has a pulse and is breathing.

EMT: [Check's the patient's pupils] Pupils are pinpoint.

Paramedic: [Administers narcan intranasally]

EMT: [Starts ventilating the patient with the BVM]

Paramedic: [Establishes an IV and administers more Narcan through the IV]

Patient: [Starts to wake up]

EMT: [Stops ventilating the patient] Ma'am, we are here with the rescue squad. We need to take you to the hospital

Patient:No no no I don't want to go to the hospital.

Patient's mom: Rachael, you almost died. You have to go to the hospital.

Patient: Fine.

EMT and Paramedic: [Help the patient onto the stretcher and into the ambulance]

EMT: [Takes vitals in the back of the ambulance which are as follows: GCS 15, 95 bpm, respirations at 12/min, BP at 120/75 mmHg, and SPO2 at 97%.]

EMT and Paramedic: [Transport the patient to the hospital without incident]

Scenario 3 - Gemini Generated - General Trauma??

A car accident involving a male in his 50s with multiple injuries. Initial assessment revealed GCS 10, agitated with altered mental status, showing labored breathing with wheezing. Initial vitals showed a pulse of 120 bpm, respiration at 22/min, BP is at 160/90 mmHg, and SPO2 at 89%.

Ventilating the patient with a BVM

Ventilating with a BVM : Start Action

...

End Action

Post-treatment, the patient experienced improved respiratory effort and vitals: GCS 15, pulse 100 bpm, BP 130/80 mmHg, respiration 18/min, and SPO2 100%. Transported to the nearest trauma center for further evaluation and treatment.

Verified and Realistic Scenario 3 - (Work in Progress)

Scenario 4 - Gemini Generated - Cardiac Arrest

Responded to an elderly female in a private residence, found unresponsive and pulseless by her neighbor. The patient is displaying abnormal breathing and no pulse.

Do 2 times:

Initiating CPR.

CPR: Start Action (**30 Compressions**)

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : Start Action (**2 Ventilations**)

...

End

After two rounds of CPR, the **AED advised a shock**. Attaching AED pads.

Attaching AED pads: Start Action

...

End

AED shock was delivered.

After 10 minutes of CPR, the patient regained a pulse and started breathing spontaneously. Transporting to the local hospital for further care and monitoring.

Verified and Realistic - Scenario 4 - Cardiac Arrest

En route

Dispatch: Medic 427 is responding to a potential cardiac rest of an 86 year old female found unconscious at home.

On scene

EMT and Paramedic: [Enter the patient's home through the unlocked front door] Hello, we are here with the rescue squad!

Patient's caretaker: Please help me in here!

[EMT and Paramedic in to the living room to find the patient unconscious on the floor with the patient's caretaker administering CPR]

EMT: [Approaches the patient] Ma'am, can you move out of the way?

EMT: [Checks patient for a pulse] No pulse and she isn't breathing. Starting CPR

Paramedic: [Over the radio to dispatch] Albemarle, Medic 427 confirms the code. The patient does not have a pulse and is not breathing. Starting CPR now.

EMT 1: [Starts compressions]

Paramedic 1: [Attaches AED to the patient]

[Another medic unit and fire truck arrive on scene]

Firefighter 1: [Begins ventilating the patient in between compressions] I'll get the BVM

Paramedic 1: [Intubates patient and Firefighter 1 takes back over ventilation after intubation] I'm going to try intubating the patient

Paramedic 2: [Establishes an IV] I will try to establish IV access

[AED advises a shock and delivers the shock. Patient regains a pulse, is still unconscious and breathes shallowly. Compressions are stopped but ventilations continue]

[The crew helps get the patient onto the stretcher, into the back of the ambulance and to the hospital where care is transferred]

Scenario 5 - Gemini Generated - Cardiac Arrest

Received a call regarding breathing difficulties and arrived to find a 70-year-old male in a hotel lobby, alert but struggling to breathe and clearly distressed. He reported a three-day history of worsening chest pressure, breathing difficulty, and abdominal pain, with no recent strenuous activity. The patient has a significant cardiac history with six stents, a coronary artery bypass, and hypertension but was unsure of his medications.

Primary assessment revealed the patient was conscious, with a rapid and irregular radial pulse, and cool, pale, moist skin. He was fully oriented. Initial examination showed an irregular pulse of 101, respiratory rate of 28, **Blood pressure is at 98/50**, oxygen saturation at 89%, and a glucose level of 87, maintaining a Glasgow Coma Scale (GCS) score of 15. An EKG indicated ST elevation and sinus tachycardia with frequent premature ventricular contractions (PVCs), suggesting acute cardiac issues.

The patient experienced severe difficulty speaking without becoming short of breath. The patient then lost consciousness and became pulseless and apneic. An oropharyngeal airway was inserted.

Starting CPR

Do 2 times:

CPR: Start Action (**30 Compressions**)

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : Start Action (**2 Ventilations**)

...

End

The EKG showed ventricular fibrillation (VF) during a brief pause in CPR.

Treatment included supplemental oxygen, 324mg chewable aspirin, and a 12-lead ECG. An intravenous line was established. Due to low blood pressure, nitroglycerin was contraindicated, but fentanyl was administered at 1mcg/kg.

The patient was urgently transported to a facility capable of cardiac catheterization. Following interventions, the patient's condition improved: pulse normalized to 96 and became regular, respiratory rate decreased to 24 with less effort, blood pressure increased to 104/61, oxygen saturation rose to 94%, and pain reduced to 4/10, with the GCS remaining at 15.

Verified and Realistic - Scenario 5 (Work in progress) - Cardiac Arrest

En route

Dispatch: Medic 423 is responding to a car crash.

On scene

EMT and Paramedic: [Find a car crashed into the top of the back of a parked car with the technical rescue unit investigating the crash]

Rescue Technician: We found one patient trapped in the crashed car. He is in his mid 30s and has a weak pulse.

[Technical Rescue Unit stabilizes the crashed car while the EMT and paramedic prepare the stretcher for the patient. Technical Rescue Unit extracts the patient and delivers the patient to the stretcher]

[Once on the stretcher, EMT checks for a pulse as the patient is moved into the back of the ambulance]

EMT: No pulse. Starting CPR

[The EMT and Paramedics administering CPR while in route to the hospital and transfer care at the hospital]

Scenario 6 - Sion - medical - supraventricular tachycardia (including atrial fibrillation) medical - tachycardia medical - ventricular tachycardia with a pulse (protocol 2 - 8)

We were dispatched to a rural highway outside city limits for a participant in a bike race who had collapsed. We arrived at the scene. We see a robust middle-aged man in cycling gear receiving bystander CPR. We access the scene safely. Hello? Sir? The patient is unresponsive to painful stimuli and has no pulse or respiration. We are taking over bystander CPR. We are administering continuous chest compressions and AED

Attaching AED pads.

Attaching AED pads: Start

...

End

Starting CPR

Do 2 times:

CPR: Start Action (30 Compressions)

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : Start Action (2 Ventilations)

...

End

After two rounds of CPR, the **AED advised a shock.**

AED shock was delivered.

The patient regained a pulse and started breathing spontaneously. Transporting to the local hospital for further care and monitoring.

Verified and Realistic - Scenario 6 - (Work in Progress)

Scenario 7 - Sion - medical - respiratory distress/asthma/copd/croup/reactive airway (respiratory distress) (protocol 3 - 11)

We were dispatched to a medical emergency in a public park. We arrived on the scene. We see an elderly, unresponsive female, and bystanders have already initiated CPR. The husband says she has a history of heart disease and had chest pain earlier during the day. Shortly before EMS arrival, around 2 minutes ago, the patient became unresponsive. The patient is pulseless and apneic Hello? and not responsive to painful stimuli. We **start CPR and defibrillation**

Attaching AED pads.

Attaching AED pads: **Start Action**

...

End Action

Starting CPR

Do 2 times:

CPR: **Start Action (30 Compressions)**

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : **Start Action (2 Ventilations)**

...

End Action

After two rounds of CPR, the **AED advised a shock.**

AED shock was delivered.

The patient regained a pulse and started breathing spontaneously. Transporting to the local hospital for further care and monitoring.

Verified and Realistic - Scenario 7 - (Work in Progress)

Scenario 8

We were dispatched to an 86-year-old woman with generalized weakness. We arrive on the scene to find the 86-year-old female patient conscious and lying on the couch. Hello, ma'am, how are you doing? Let me take your vitals. After we take her vitals, she is soaking in a cold sweat and barely conscious. She cannot support her airway. She had a medical history of MI 15 years ago.

Her **blood pressure is 100 over 60**. Her pulse is 114, respiration is 30 per minute. Her skin is cool, clammy, and pale. Her temperature is 96.8. O2 saturation is 98%. We prop her up on several pillows to keep her head in an elevated position. **We assist with BVM and O2.**

Attaching Oxygen mask: **Start Action**

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : **Start Action**

...

End Action

The patient began to breathe on her own without assistance and vital signs improved.

Remove Oxygen mask: **Start Action**

...

End Action

Scenario 9

Upon arriving at the scene of a cardiac arrest, we promptly assessed the patient, a 72-year-old male was found unresponsive in his home. After confirming unresponsiveness, we are **initiating CPR and attaching AED pads.**

Attaching AED pads.

Attaching AED pads: **Start Action**

...

End Action

A shockable rhythm was detected. We cleared the patient and delivered the shock.

Continuing CPR as per the AED prompts.

Do 2 times:

CPR: **Start Action (30 Compressions)**

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : **Start Action (2 Ventilations)**

...

End Action

Our coordinated efforts contributed to restoring the patient's pulse and regaining consciousness. We are transporting him to the hospital.

Scenario 10

We were dispatched to a 68-year-old male with shortness of breath. We arrived on the scene, and we saw the 68-year-old male conscious and lying on the side of the bed, gasping for air. The patient is anxious and has trouble supporting his own airway.

Can I take your vitals? His vital signs are blood pressure of 196 over 100, pulse of 116, and respiration of 36 per minute. Skin is cool, clammy, and pale. O2 of 92 percent. We keep his head in an elevated position and **assist with BVM** and O2. We transport the patient to the hospital.

Attaching Oxygen mask: **Start Action**

...

End Action

Ventilating with a BVM with 100% Oxygen.

Ventilating with a BVM : **Start Action**

...

End Action

The patient began to breathe on her own without assistance, and vital signs improved.

Remove Oxygen mask: **Start Action**

...

End Action
